

Name : DUMMY
Lab No. : Z6152301
Ref By : DR. DUMMY DUMMY
Collected : 23/7/2024 6:39:00AM
A/c Status : P
Collected at : PRODUCTION TEST COLLECTION CENTRE
SECTOR - 18, BLOCK-E ROHINI
DELHI 110085

Age : 25 Years
Gender : Male
Reported : 23/7/2024 6:47:33PM
Report Status : Interim
Processed at : LPL-NATIONAL REFERENCE LAB
National Reference laboratory, Block E,
Sector 18, Rohini, New Delhi -110085



Test Report

Test Name	Results	Units	Bio. Ref. Interval
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HEART HEALTH SCREEN, ADVANCED

APOLIPOPROTEIN B (Apo B) (Immunoturbidimetry)	46.00	mg/dL	46 - 174
GLUCOSE, FASTING (F), PLASMA (GOD POD)		mg/dL	70 - 100
CARDIO C-REACTIVE PROTEIN (hsCRP), SERUM (Immunoturbidimetry)	1.00	mg/L	<1.00

Interpretation

CARDIO CRP IN mg/L	CARDIOVASCULAR RISK
<1	Low
1-3	Average
3-10	High
>10	Persistent elevation may represent Non cardiovascular inflammation

Note: To assess vascular risk, it is recommended to test hsCRP levels 2 or more weeks apart and calculate the average

Comments

High sensitivity C Reactive Protein (hsCRP) significantly improves cardiovascular risk assessment as it is a strongest predictor of future coronary events. It reveals the risk of future Myocardial infarction and Stroke among healthy men and women, independent of traditional risk factors. It identifies patients at risk of first Myocardial infarction even with low to moderate lipid levels. The risk of recurrent cardiovascular events also correlates well with hsCRP levels. It is a powerful independent risk determinant in the prediction of incident Diabetes.

LIPOPROTEIN(a); Lp(a), SERUM (Immunoturbidimetry)	mg/dL	<20
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HEART HEALTH SCREEN, ADVANCED

TROPONIN- I, SERUM HIGH SENSITIVE (CMIA)	4	ng/L
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Recommended hsTnI risk stratification cut-off values for asymptomatic individuals using ARCHITECT HsTnI assay

HsTrop I levels in Male	HsTrop I levels in Females	CVD Risk Category	Interpretation and Advice
<6	<4	Low Risk	Cardiac injury unlikely, Follow up every year
6-12	4-10	Moderate Risk	If there is no other risk factor or comorbidity reassess every 6/12 months. Lifestyle modification recommended
>12	>10	High Risk	Cardiac injury likely, rule out strenuous exercise in past 3 days and/or renal dysfunction interference. Discuss results with treating clinician for further management

Reference: Consensus Statement on the Use of High-Sensitivity Troponin I Assay for Risk Stratification of Apparently Healthy Individuals - An Indian dimension", *International Journal of Current Research*, 13, (01), 15771-15778

Note

- Any condition resulting in myocardial injury can potentially increase hsTnI levels thus the results should be used in conjunction with other information such as ECG, clinical observations & symptoms, etc.
- False positive results can be seen in the presence of Rheumatoid factor and heterophile antibodies

Comment

This assay is recommended for cardiovascular disease (CVD) risk stratification in asymptomatic individuals. High-sensitivity troponin I (hsTnI) assays have the ability to detect very low concentrations of circulating cardiac troponin even in apparently normal and healthy individuals.



Name :	DUMMY	Age :	25 Years
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	DELHI 110085		



Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIPID PROFILE, BASIC, SERUM			
Cholesterol Total (CHO-POD)		mg/dL	<200
Triglycerides (GPO-POD)		mg/dL	<150
HDL Cholesterol (Enz Immunoinhibition)		mg/dL	>40
LDL Cholesterol, Direct (Enz Selective protection)		mg/dL	<100
LDL Cholesterol (Calculated)		mg/dL	<100.00
VLDL Cholesterol (Calculated)		mg/dL	<30
Non-HDL Cholesterol (Calculated)		mg/dL	<130

Note

1. Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
2. Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors

ASCVD Risk Stratification & Treatment goals in Indian population

1. Indians are at very high risk of developing ASCVD, they usually get the disease at an early age, have a more severe form of the disease and have poorer outcome as compared to the western populations
2. Many individuals remain asymptomatic before they get heart attack, ASCVD risk helps to identify high risk individuals even when there is no symptom related to heart disease
3. ASCVD risk category helps clinician to decide when to consider therapy and what should be the treatment goal



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Treatment Goals as per Lipid Association of India 2020

ASCVD RISK CATEGORY@	CONSIDER THERAPY		TREATMENT GOAL	
	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)
Extreme (A)	>=50	>=80	<50 (Indispensable) <30 (Optional)	<80
Extreme (B)	>=30	>=60	<30	<60
Very High	>=50	>=80	<50	<80
High	>=70	>=100	<70	<100
Moderate	>=100	>=130	<100	<130
Low	>=130*	>=160*	<100	<130

* In low risk patient, consider therapy after an initial non-pharmacological intervention for at least 3 months

@To know your risk category click on bit.ly link sent on your registered mobile number, answer the questionnaire, the ASCVD risk report can be downloaded from website



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Test Name	Results	Units	Bio. Ref. Interval
HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD (HPLC, NGSP certified)			
HbA1c		%	4.00 - 5.60
Estimated average glucose (eAG)		mg/dL	

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults ≥ 18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	≥ 6.5	< 7.0

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.

FACTORS THAT INTERFERE WITH HbA1c MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1c RESULTS
Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c

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Result/s to follow:

TROPONIN- I, SERUM HIGH SENSITIVE, LIPID PROFILE, BASIC, SERUM, HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD, GLUCOSE, FASTING (F), PLASMA, LIPOPROTEIN(a); Lp(a), SERUM

IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory .
•Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician .•Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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National Reference lab, Delhi, a CAP (7171001) Accredited, ISO 9001:2015 (FS60411) & ISO 27001:2013 (616691) Certified laboratory.

